



Dietetics Referral Pathway – Emergency Department Procedure

1. Scope

Site	Operational Area	Applicable to
Armadale Kalamunda Group	Emergency Department	Allied Health, Nursing and Medical

2. Purpose

The purpose of this procedure is to assist the Emergency Department (ED) allied health, nursing and medical teams to complete appropriate referrals to dietetics.

3. Procedure

To determine if your patient requires urgent dietetic input in the ED or as an inpatient (e.g. issues with enteral feeds, unable to tolerate oral intake), refer to [Appendix 1](#) Indications for Referral to Dietetics. During business hours 0800-1630 Monday – Friday, contact the dietetics department on ext 2361 or relevant dietitian on page:

- Campbell #148
- Canning #225
- Colyer #039
- Intensive Care #226
- Medical Admission Unit #148
- Rehabilitation #259

If an inpatient referral is required after hours or on weekends leave a message on ext 2361 or email ahs.dietetics@health.wa.gov.au with details.

Handover details can be entered into iSOFT under 'HI Nut&DietDept', however this is to be in conjunction with above referral process.

If the patient is for discharge home, complete referral to outpatient dietetic services, refer to [Appendix 1](#) Indications for Referral to Dietetics and [Appendix 2](#) Dietetic Outpatient Referral Options.

4. Legislative context

The following documents are required to give effect to this procedure:

- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP 0095](#)
- HDWA [Specialist Outpatient Services Access Policy](#)

5. Supporting information

The following documents support the implementation of this procedure:

- [Outpatient Referral Triage and Management Guidelines AKG-GUI-0565-21](#)

6. Compliance, monitoring and evaluation

Compliance against this procedure will be evaluated by appropriate referrals made to dietetics department.

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.28 Nutrition and hydration
- The workforce uses the systems for preparation and distribution of food and fluids to:
 - a. Meet patients' nutritional needs and requirements
 - b. Monitor the nutritional care of patients at risk
 - c. Identify, and provide access to, nutritional support for patients who cannot meet their nutritional requirements with food alone
 - d. Support patients who require assistance with eating and drinking
- Standard 6 – Clinical handover
- Clinicians use structured clinical handover processes that include:
 - a. Preparing and scheduling clinical handover
 - b. Having the relevant information at clinical handover
 - c. Organising relevant clinicians and others to participate in clinical handover
 - d. Being aware of the patient's goals and preferences
 - e. Supporting patients, carers and families to be involved in clinical handover, in accordance with the wishes of the patient
 - f. Ensuring that clinical handover results in the transfer of responsibility and accountability for care

8. Policy owner (relating to these procedures)

Director Allied Health

Enquiries relating to this procedure may be directed to:

Title: Coordinator Dietetics
Department: Dietetics
Email: jane.priest@health.wa.gov.au

9. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	7/11/2024	7/11/2027	Scheduled review – minor amendments

10. Authorisation

This procedure has been authorised and issued by the Director Allied Health and Ambulatory Care.

Authorised by	Danielle Kilmurray – Director Allied Health and Ambulatory Care
Authorisation date	22/11/2024
Published date	27/11/2024

Appendix 1 Indications for referral to dietetics

	Clinical indicator
Bariatric Risk Assessment	BMI >35 Note: outpatient weight management referrals not accepted at AKG
Cardiac Disease	Fluid overload – new
Diabetes	Newly diagnosed Poorly controlled Having hypo's
Dysphagia	Requiring texture modified diet/fluids – often require nutritional support
Eating Disorders	Anorexia Nervosa Bulimia Note: outpatient referrals not accepted at AKG
Enteral feeds –	Issues with PEG, PEJ, NGT – if sole source of nutrition will require inpatient admission Dysphagia requiring insertion of NGT
Food Allergies / Intolerances	Newly diagnosed
Gastrointestinal Conditions	Prolonged diarrhoea Constipation – if linked to presenting complaint Ulcerative colitis/Crohn's disease – if linked to presenting complaint indicate a flare up Coeliac disease – if newly diagnosed Bowel obstruction – if linked to presenting complaint High output stomas
Liver Disease	Encephalopathy Ascites Hypoalbuminaemia/low albumin Drowsy and unable to take adequate amounts orally
Malnutrition or at risk of malnutrition	Low body weight (BMI <18) Weight loss >5kg in past month Prolonged nausea and vomiting Taking oral nutrition support products at home (e.g. Ensure, Sustagen, Resource) Oncology patients especially head and neck, upper GI and those receiving treatment Parkinson's disease Huntington's disease Chronic Pancreatitis- if linked to presenting complaint Growth faltering in paediatrics
Renal Failure	Hyperkalaemia – potassium >5.9 Hyperphosphataemia – high phosphate level On dialysis New to dialysis unit/commencing dialysis
Wound Healing	Pressure injuries > stage 2, poor wound healing

Appendix 2 Dietetics outpatient referral options

Community Rehabilitation (CR)	Dietetics General Outpatients	Dietetics Diabetes Outpatients (including Gestational Diabetes)	Dietetics Paediatric Outpatients	External Primary Care Services
If patient is already known to CR or requires multidisciplinary team input ? Does the patient have diagnosed cardiac failure or Chronic Obstructive Pulmonary Disease or any other diet related issue ? Complete Community rehabilitation referral via e-Referrals https://ereferrals.hdwa.health.wa.gov.au/ In 'Unit' field select Rehabilitation Medicine [RHAB]	Is the referral going to prevent an inpatient admission? Has the patient been an inpatient to AHS within the last 12 months? Does the referral meet the Dietetic Outpatient Clinical Prioritisation Guide as per the AKG Dietetics Non-Admitted Patient Procedure? Complete OP Dietetics referral via e-Referrals https://ereferrals.hdwa.health.wa.gov.au/ In 'Unit' field select Dietetics [DIE] then Dietetics-General [DIGE]	Does the patient have diabetes? Does the patient meet the AKG Diabetes Outpatient Referral Criteria, https://akg-healthpoint.hdwa.health.wa.gov.au/directoryAKG/ClinicalServices/diabetes/Pages/default.aspx ? Complete Diabetes Education Referral via e-Referrals https://ereferrals.hdwa.health.wa.gov.au/ In 'Unit' field select Diabetes [DIA] then choose the relevant option	Is the referral going to prevent an inpatient admission? Does the referral meet the Dietetic Paediatric Outpatient Procedure? Complete OP Dietetics referral via e-Referrals https://ereferrals.hdwa.health.wa.gov.au/ In 'Unit' field select Dietetics [DIE] then Paediatrics [DIPA]	Does the patient require dietetic input for hyperlipidaemia, hypertension, overweight/obesity, diabetes and does not meet AKG referral criteria? Refer to GP for Chronic Disease Management Plan or Primary Health Care Service (e.g. 360 Health + Community) referral. Further information available via the Localised directory of services, https://www.ahs.health.wa.gov.au/Health-Professionals/Localised-Directory-of-Services Patients with Eating Disorders not admitted? Refer to GP for Mental Health Care Plan and referral to private dietitian